

The Global Burden of Mental Illness

Prevalence, Impact, and the Treatment Gap

Data source: IHME, Global Burden of Disease (2025) | Our World in Data (2023)

A data-driven overview for public health policy analysts and mental health program planners

Hundreds of Millions Affected

1 in 3

women will experience major depression
in their lifetime

1 in 5

men will experience major depression
in their lifetime

Why this matters for policy

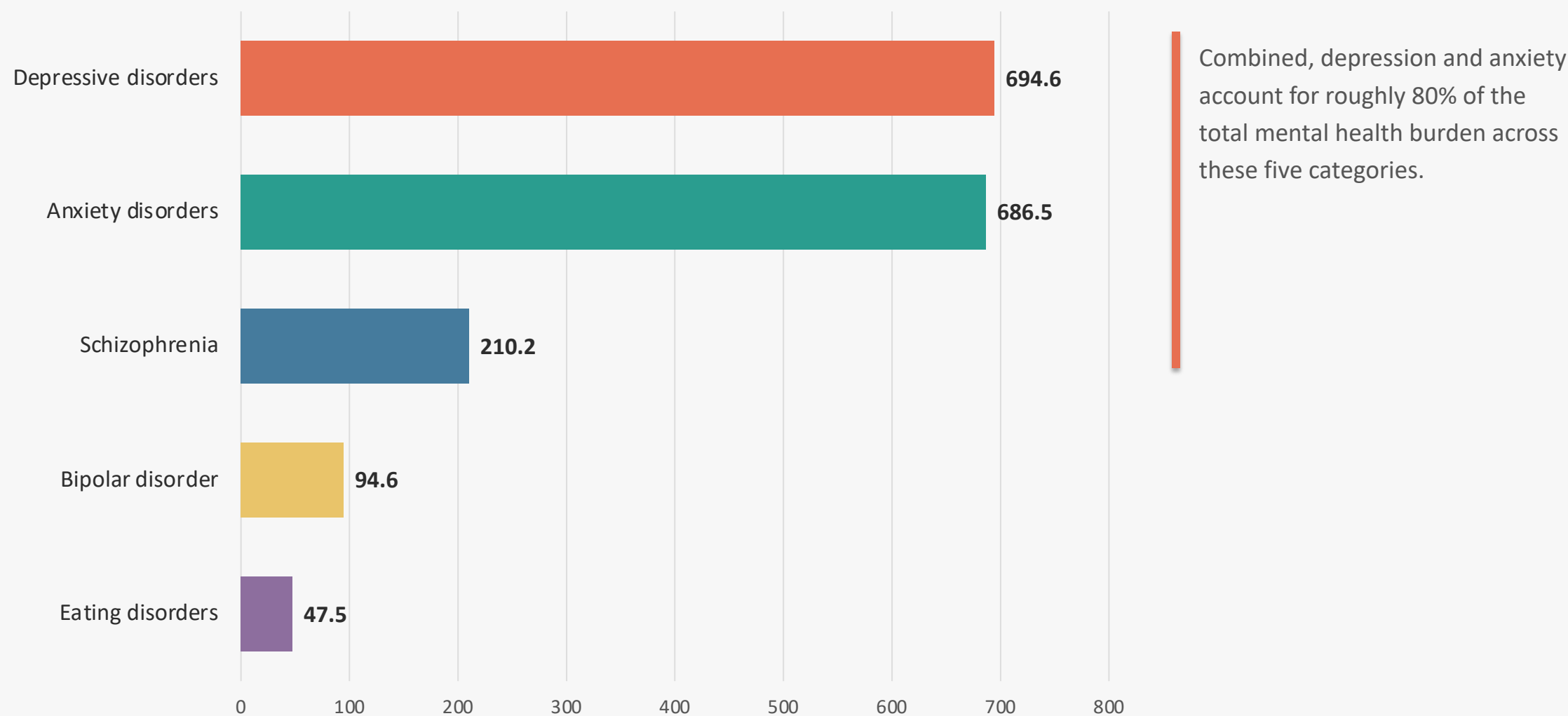
Mental health conditions span from common disorders (depression, anxiety) to less common but severe conditions (schizophrenia, bipolar disorder).

Hundreds of millions of people suffer each year, with effects on well-being, work, and relationships.

Policy responses must address both the scale of common disorders and the severity of rare ones.

Depression and Anxiety Dominate

DALYs per 100,000 people by mental illness category, World, 2023



Source: IHME, Global Burden of Disease (2025). Figure reference: figure_1

Depression and Anxiety Carry Nearly Equal Disease Burden

DALYs per 100,000 people — World, 2023

694.6

686.5

Only
8.1
DALYs
apart

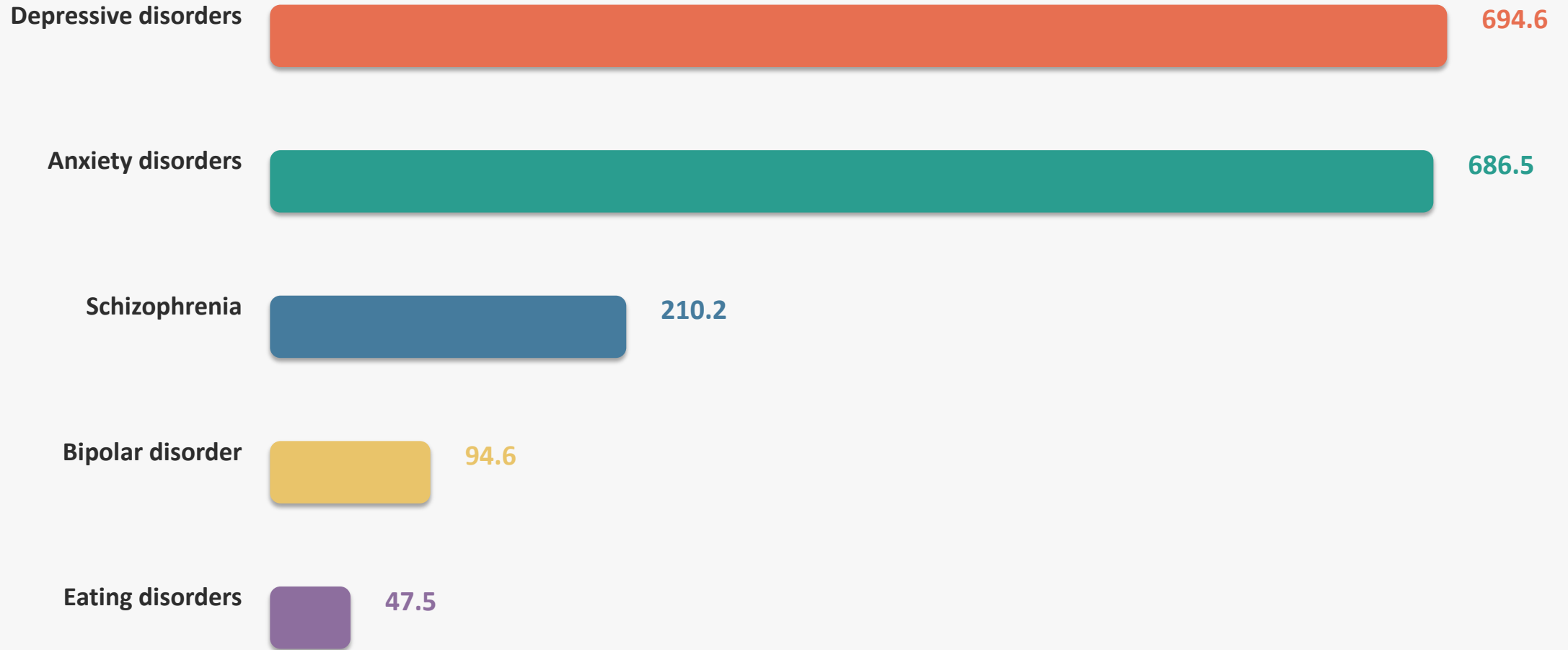
Together they dwarf all
other mental illness
categories combined.

Depressive Disorders

Anxiety Disorders

Schizophrenia and Bipolar Disorder

Lower prevalence, severe individual impact



While population-level burden is lower for schizophrenia and bipolar disorder, individual severity and disability are disproportionately high.

Treatment Is Available but Often Lacking or Poor Quality

Barriers to effective mental health care



S

Stigma

Many people are uncomfortable disclosing symptoms to healthcare professionals or even to people they know.



A

Access

Treatment is often lacking, especially in low- and middle-income countries.



Q

Quality

Even where treatment exists, it may be of poor quality.



U

Under-reporting

Discomfort with disclosure makes it difficult to estimate the true prevalence, meaning the actual burden may be higher than reported.

Mental illnesses are treatable and their impact can be reduced. Good data is essential to understand these conditions — how, when, and why they occur, how many people are affected, and how they can be treated effectively and safely.

How People Cope

From medication to faith to family conversations



M

Prescribed Medication

Common in higher-income settings where healthcare infrastructure and access to pharmaceuticals are more readily available.



R

Religious / Spiritual Activities

Prevalent in many regions worldwide. Faith-based coping is a significant response to anxiety and depression across cultures.



T

Talking to Friends & Family

A widely reported coping mechanism across countries, reflecting the universal role of social support in mental health.

The diversity of coping strategies underscores that mental health responses are shaped by cultural, economic, and healthcare system factors. Policy interventions must account for these differences rather than applying a one-size-fits-all approach.

Key Takeaways

Prioritizing Depression, Anxiety, and Closing the Treatment Gap

- 1 Depression and anxiety together represent the overwhelming majority of mental health DALYs — each generating ~690 per 100,000 people.
- 2 Schizophrenia and bipolar carry disproportionate individual burden relative to their prevalence, demanding specialized care pathways.
- 3 Effective treatments exist, but access and quality remain inadequate — the treatment gap is a policy failure, not a knowledge gap.
- 4 Stigma suppresses both help-seeking and accurate data, meaning the true burden is likely higher than reported figures suggest.
- 5 Policy should prioritize scaling evidence-based interventions for common mental disorders while maintaining specialized care for severe conditions.